

# **Healthcare Data Analysis: Patient, Treatment, and Cost Insights**

Exploratory Data Analysis and Prescription Text Processing

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# Agenda

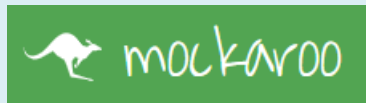
- Project Motivation & Goals
- Datasets Overview
- Methodology
  - Exploratory Data Analysis (EDA)
  - Unstructured Text Processing
- Key Findings
- Conclusions
- Future Work

# Project Motivation

- Healthcare systems generate large amounts of patient and treatment data
- Understanding patterns can help:
  - Improve resource allocation
  - Identify cost drivers
  - Analyze treatment effectiveness

**WHY THIS  
PROJECT?**

- **1<sup>st</sup> Objective: Dataset for analysis**



<https://www.kaggle.com/>

# Project Goal

- Extract meaningful insights about COSTS from healthcare data
- Identify cost drivers



# Datasets Overview

<https://www.kaggle.com/>

## HealthcareDataset1 (Patients info)

- PatientID,
- PatientName
- Age
- Gender
- Bloodtype
- Diagnosis
- Treatment
- Admission Date
- Discharge Date
- Total Bill (\$)
- Full Prescription Details  
(unstructured)

# 1000 rows, 11 columns

## HealthcareDataset2 (Hospital info)

- PatientID
- Hospital
- DoctorName
- Room Number
- Daily Cost (\$)
- TreatmentType
- RecoveryRating

# 1000 rows, 7 columns

# Methodology

## Data Analytics Pipeline



# Exploratory Data Analysis

## Data Quality Check, Cleaning & Data Transformation

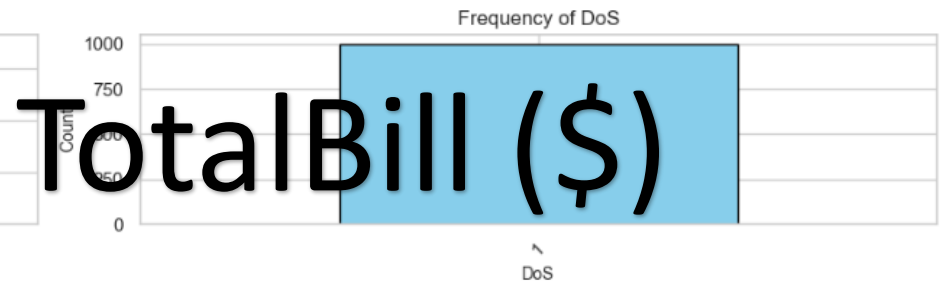
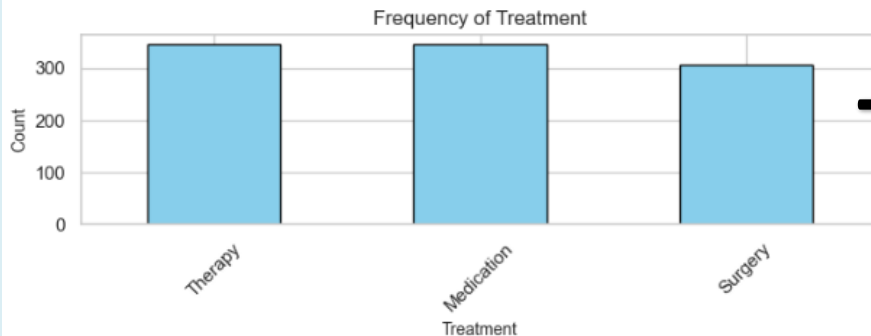
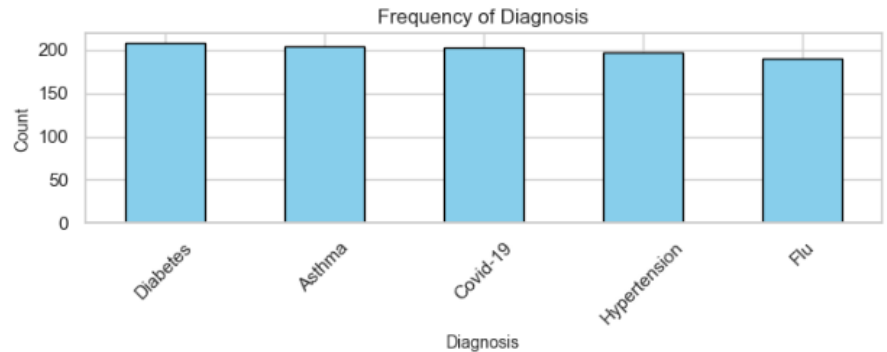
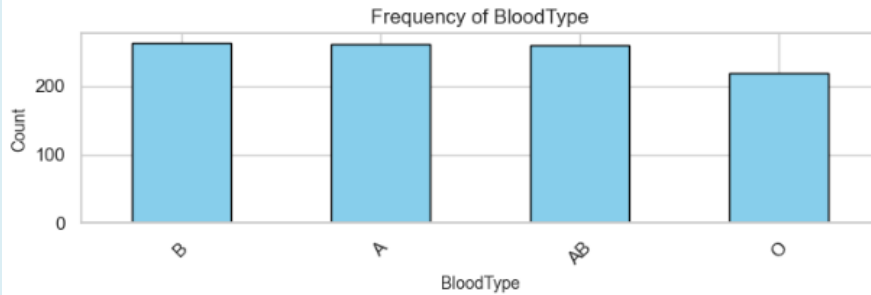
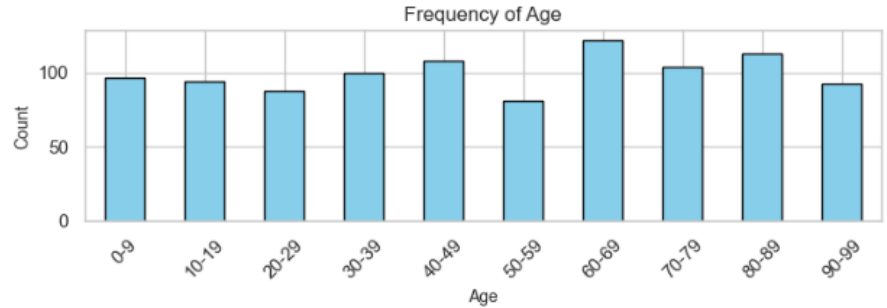
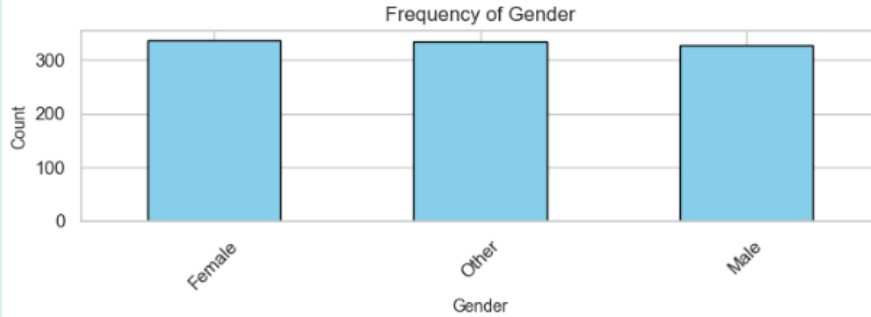
- Date consistencies
- Identify Nulls
  - Patients (6% TotalBill)
  - Doctors (7,8% RecoveryRate)
- Identify Duplicates
- Identify Outliers
- Extract information from the “Full Prescription Details” (patients)
- DoS (Days of Stay)
- Temporary Analysis
  - AdmissionDate
    - AD\_Year
    - AD\_Quarter
    - AD\_Month
    - AD\_Day
  - DischargeDate
    - DD\_Year
    - DD\_Quarter
    - DD\_Month
    - DD\_Day
- Combined dataset (PatientID):



df = clinical information + hospital operational data (# 1000 rows, 32 variables)

# Patients Breakdown

Frequency Distribution of Patients Data

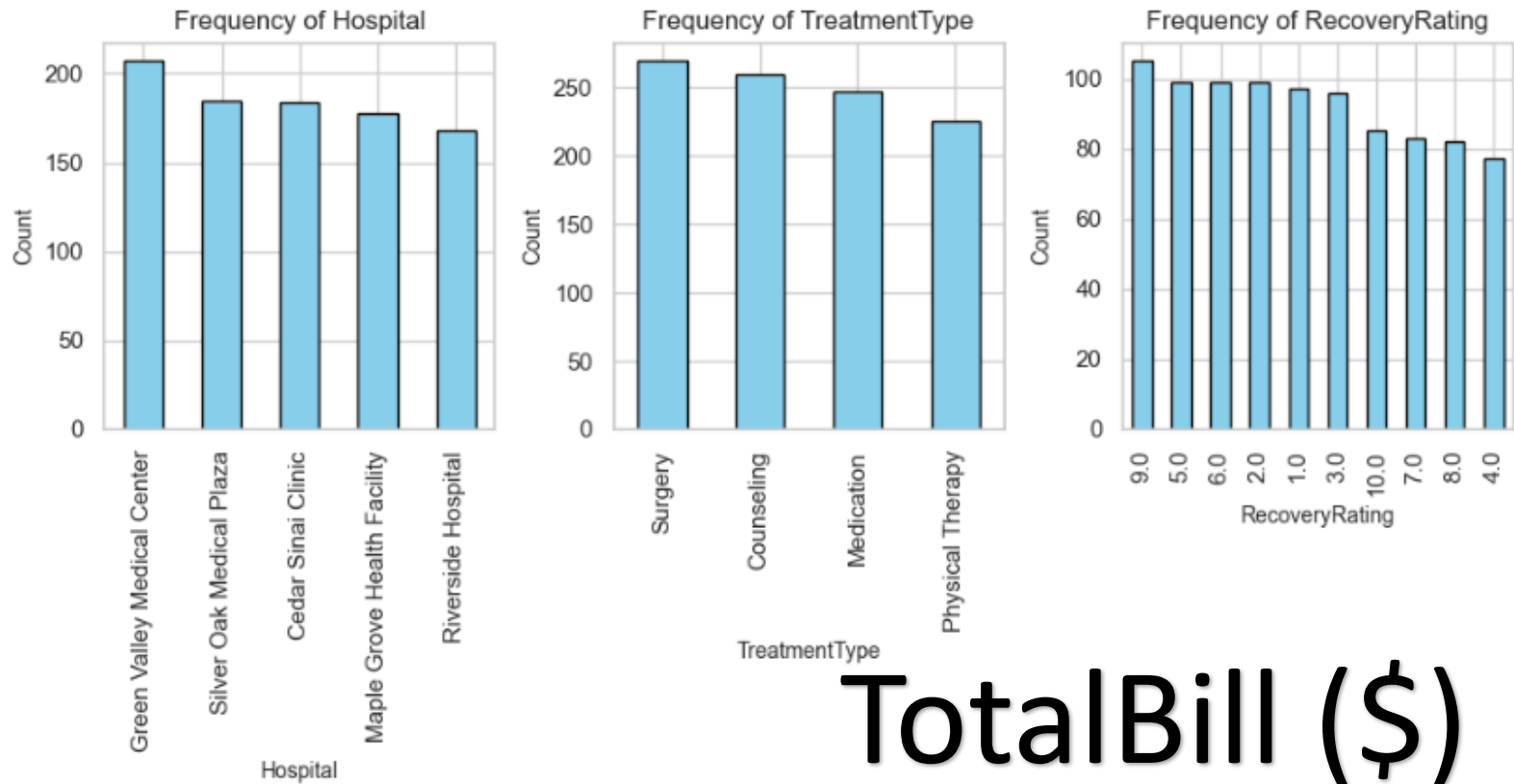


Total Bill (\$)



# Hospital Breakdown

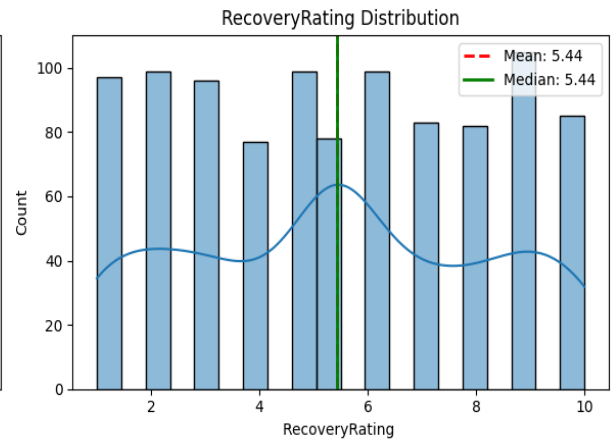
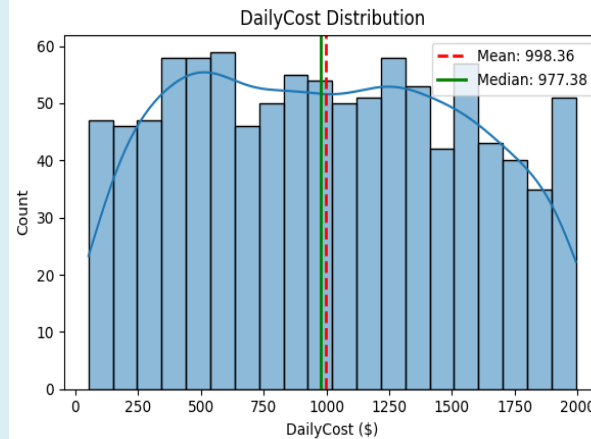
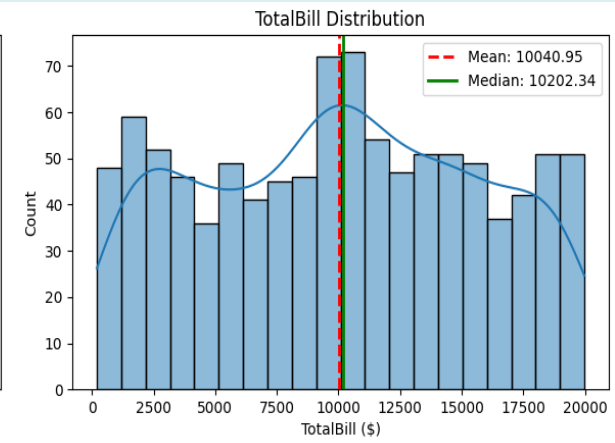
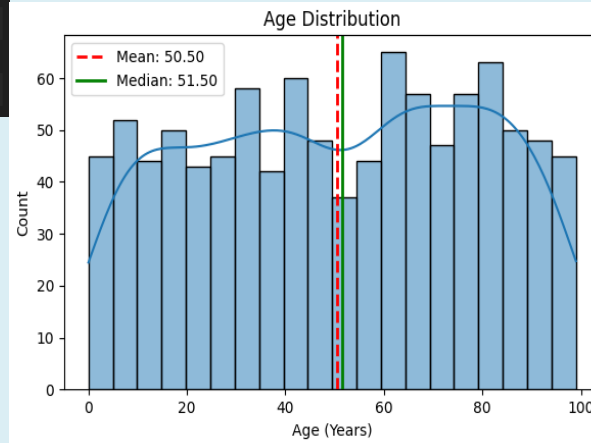
Frequency Distribution of Hospital Data



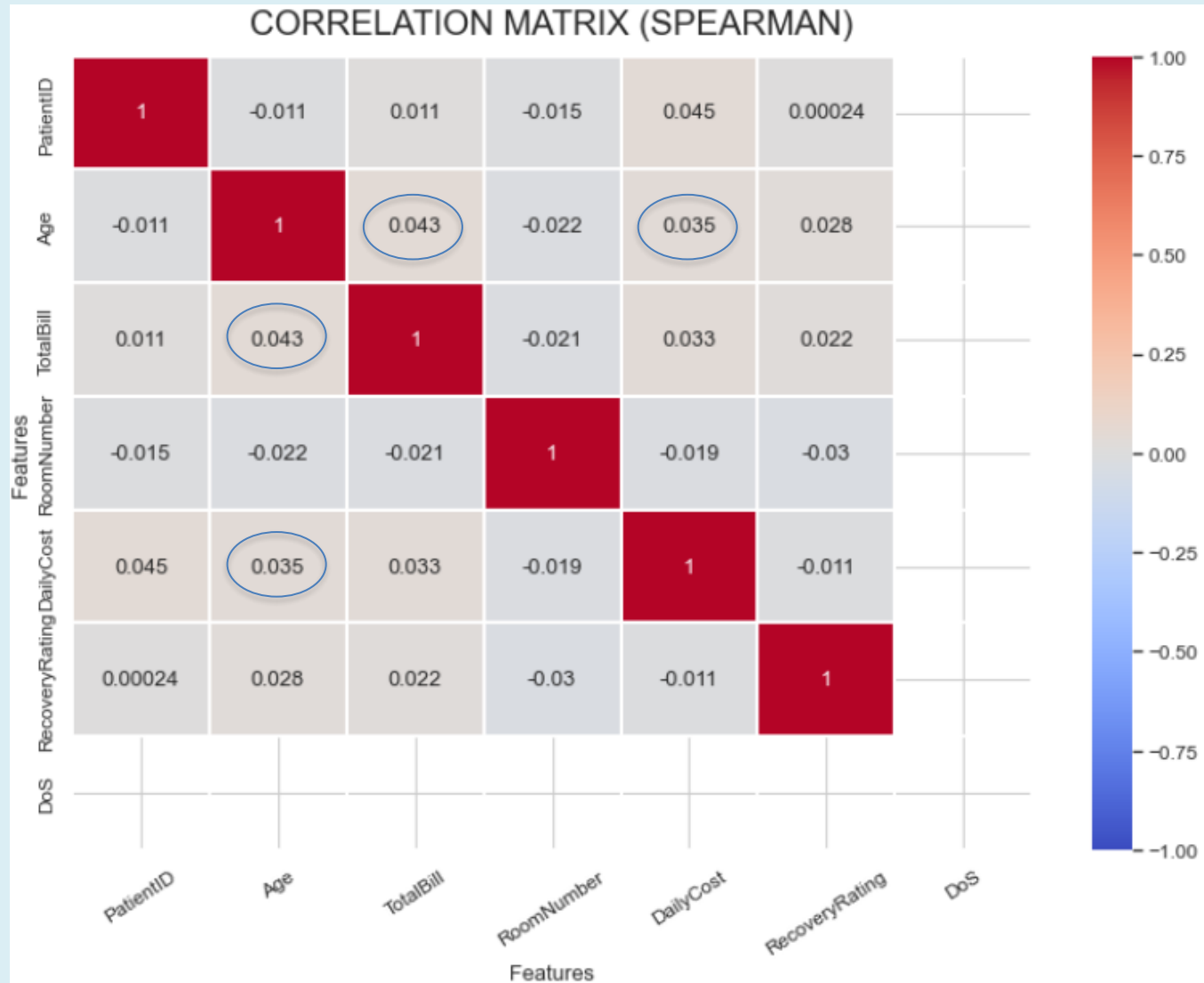
# Descriptive Statistics & Variable Distribution

|       | Age         | TotalBill    | DailyCost   | RecoveryRating | DoS    |
|-------|-------------|--------------|-------------|----------------|--------|
| count | 1000.000000 | 1000.000000  | 1000.000000 | 1000.000000    | 1000.0 |
| mean  | 50.500000   | 10040.947498 | 998.358349  | 5.436009       | 1.0    |
| std   | 28.599859   | 5626.658910  | 545.178933  | 2.776767       | 0.0    |
| min   | 0.000000    | 200.928022   | 52.023698   | 1.000000       | 1.0    |
| 25%   | 26.000000   | 5326.999625  | 527.573516  | 3.000000       | 1.0    |
| 50%   | 51.500000   | 10202.339277 | 977.376355  | 5.436009       | 1.0    |
| 75%   | 75.000000   | 14552.941322 | 1459.068550 | 8.000000       | 1.0    |
| max   | 99.000000   | 19979.201527 | 1994.047594 | 10.000000      | 1.0    |

Normal Distributions???



# Quantitative Variables Correlation



# Statistical Test Results

## Significant Differences and Associations

(T-test and Chi<sup>2</sup>)

Division variable: Age

T-statistic: -2.280, p-value: 0.023

→ There is a statistically significant difference in 'TotalBill' between the two groups based on the median of the variable.

Division variable: DailyCost

T-statistic: -0.828, p-value: 0.408

→ No significant difference in 'TotalBill' between the groups based on the median of the variable.

Division variable: RecoveryRating

T-statistic: -0.147, p-value: 0.883

→ No significant difference in 'TotalBill' between the groups based on the median of the variable.

Variable: Gender

Chi2: 1915.7667, p-value: 0.4330

→ No significant relationship with TotalBill

Variable: BloodType

Chi2: 2860.0548, p-value: 0.4909

→ No significant relationship with TotalBill

Variable: Diagnosis

Chi2: 4000.0000, p-value: 0.0167

→ There is a significant relationship with TotalBill

Variable: Treatment

Chi2: 2000.0000, p-value: 0.0657

→ No significant relationship with TotalBill

Variable: Hospital

Chi2: 3527.6197, p-value: 0.4986

→ No significant relationship with TotalBill

Variable: DoctorName

Chi2: 350587.0304, p-value: 0.5552

→ No significant relationship with TotalBill

Variable: TreatmentType

Chi2: 2863.2094, p-value: 0.4743

→ No significant relationship with TotalBill

Variable: Medicine1

Chi2: 27663.8823, p-value: 0.4534

→ No significant relationship with TotalBill

Variable: Medicine2

Chi2: 27652.0842, p-value: 0.4733

→ No significant relationship with TotalBill

Variable: Medicine3

Chi2: 18929.1735, p-value: 0.4553

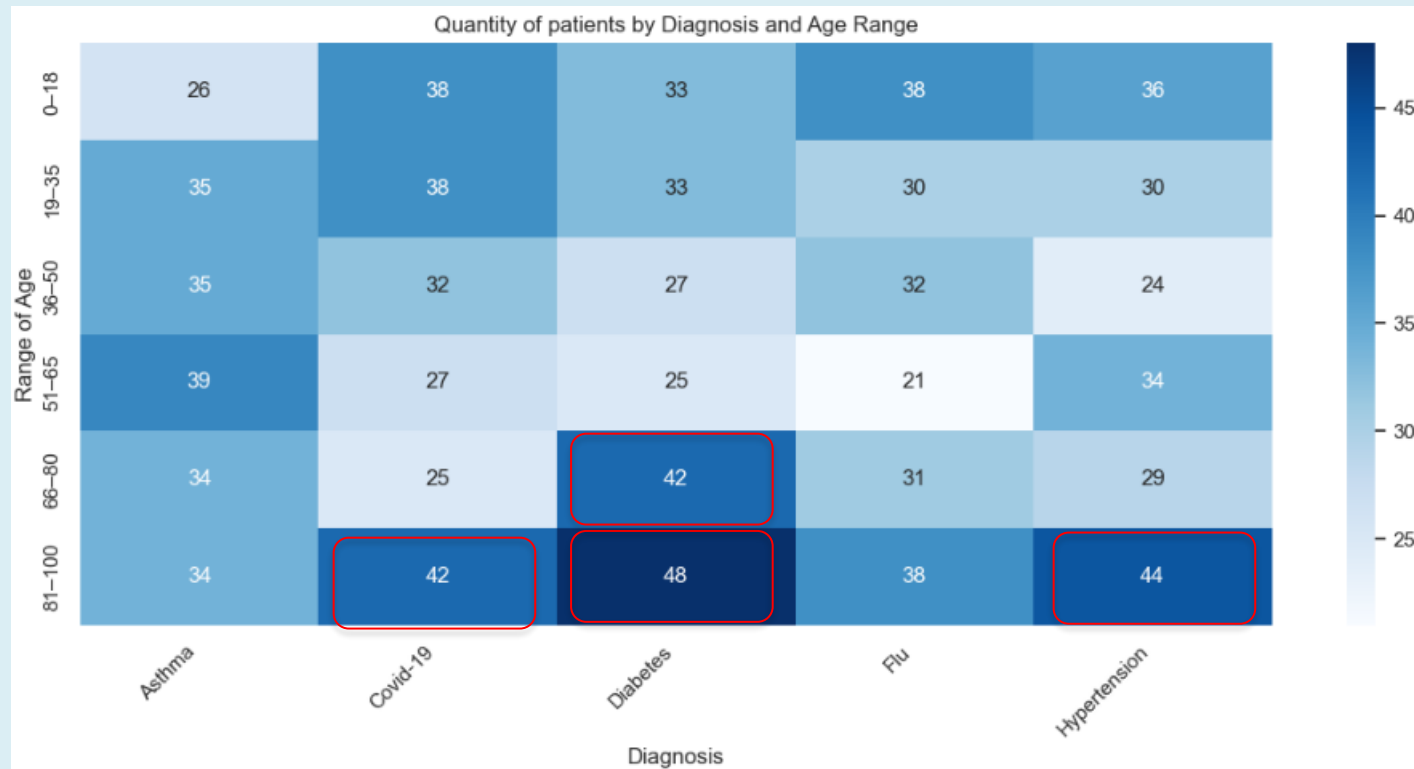
→ No significant relationship with TotalBill

Variable: Medicine4

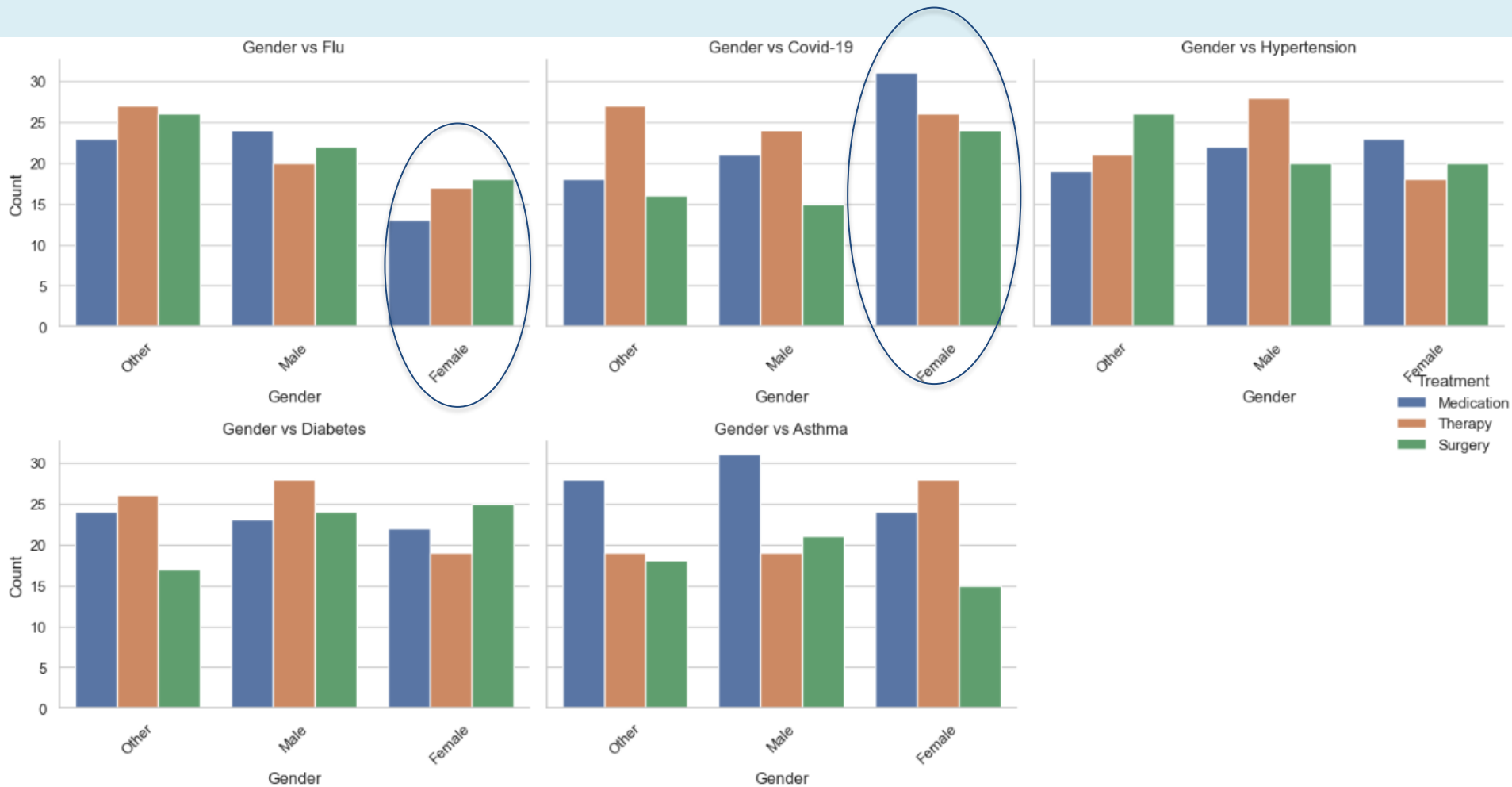
Chi2: 10147.4854, p-value: 0.4244

→ No significant relationship with TotalBill

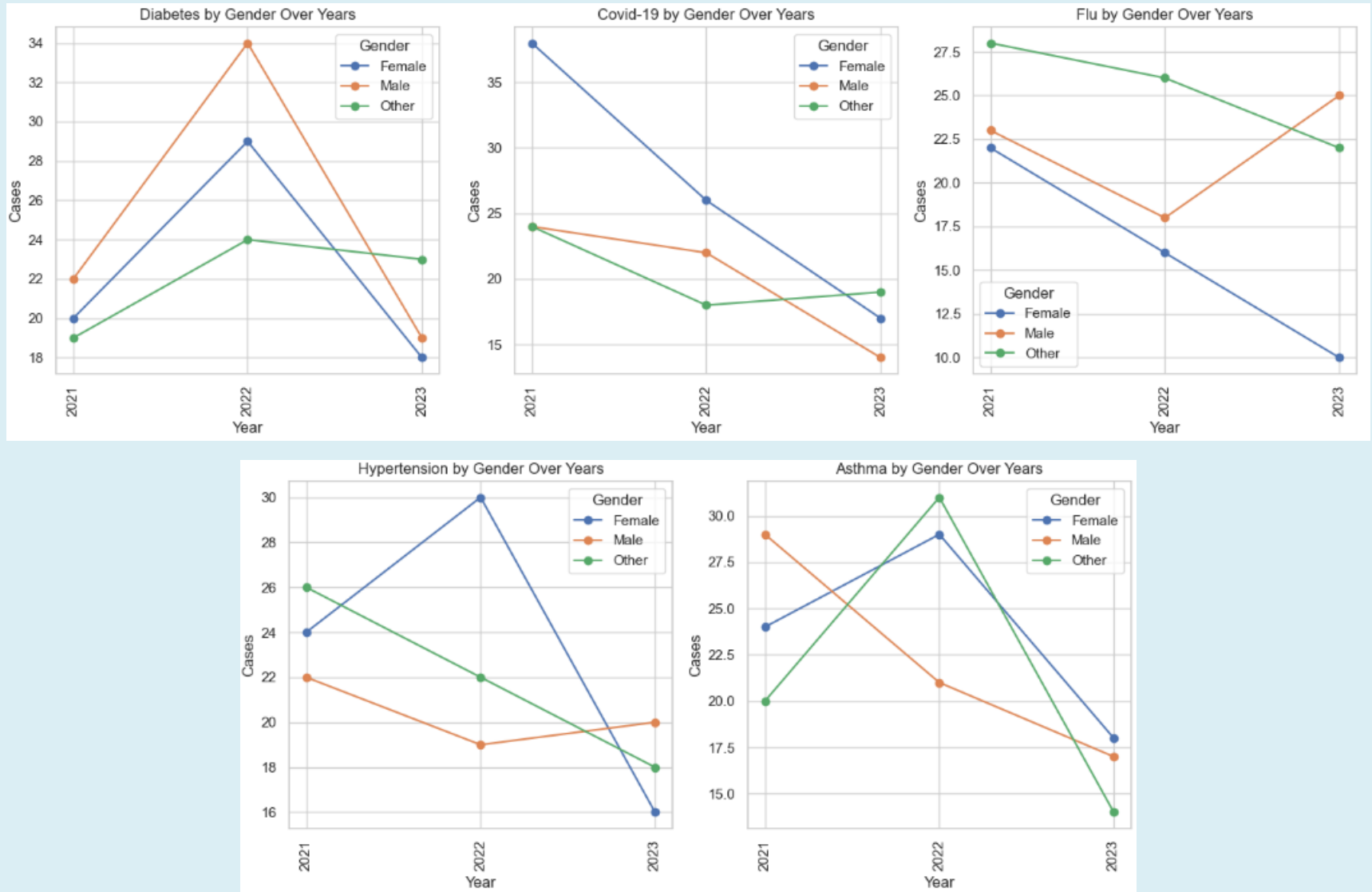
# Key Findings: Diagnosis per Age



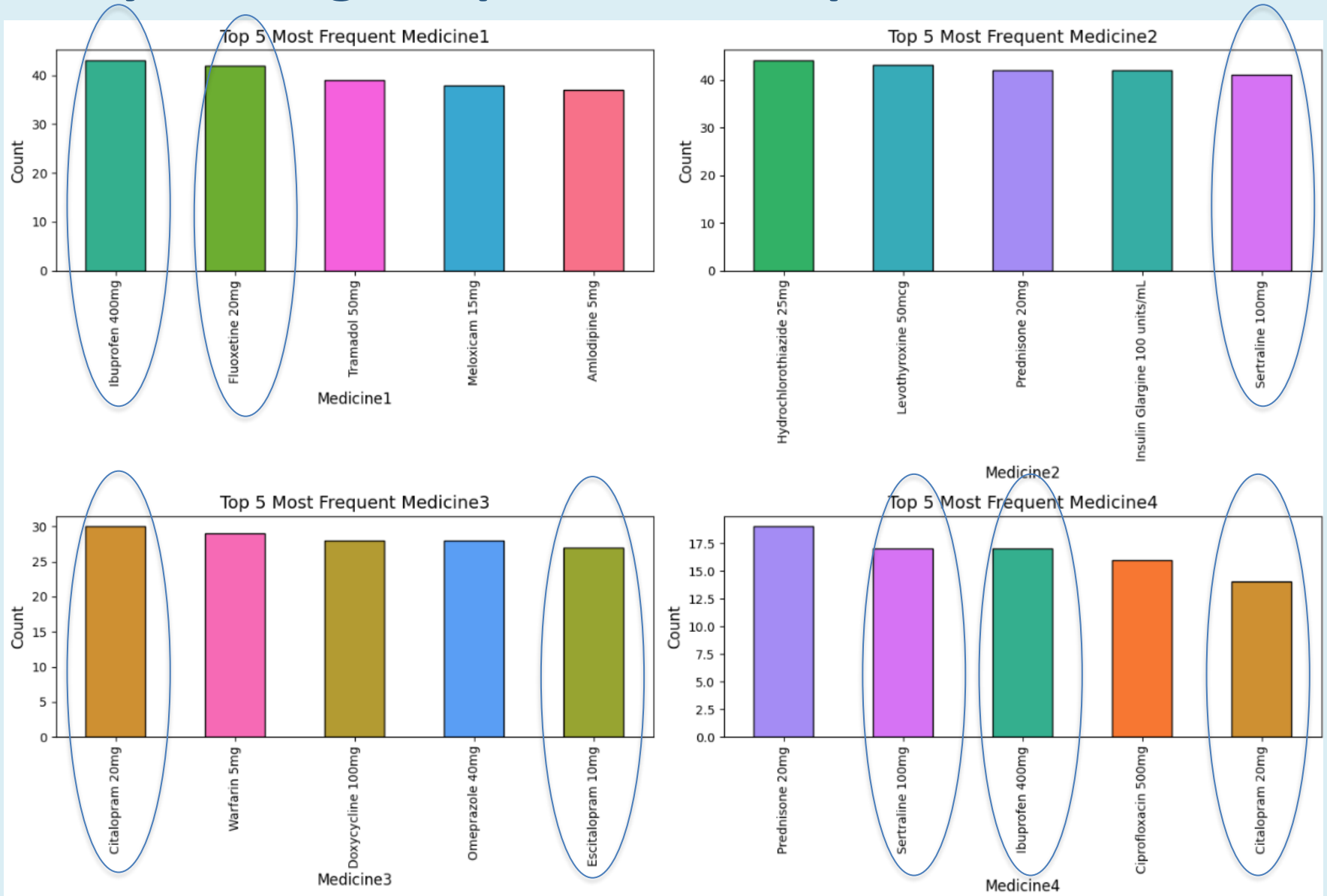
# Key Findings: Diagnosis per Gender



# Key Findings: Diagnosis by gender over years

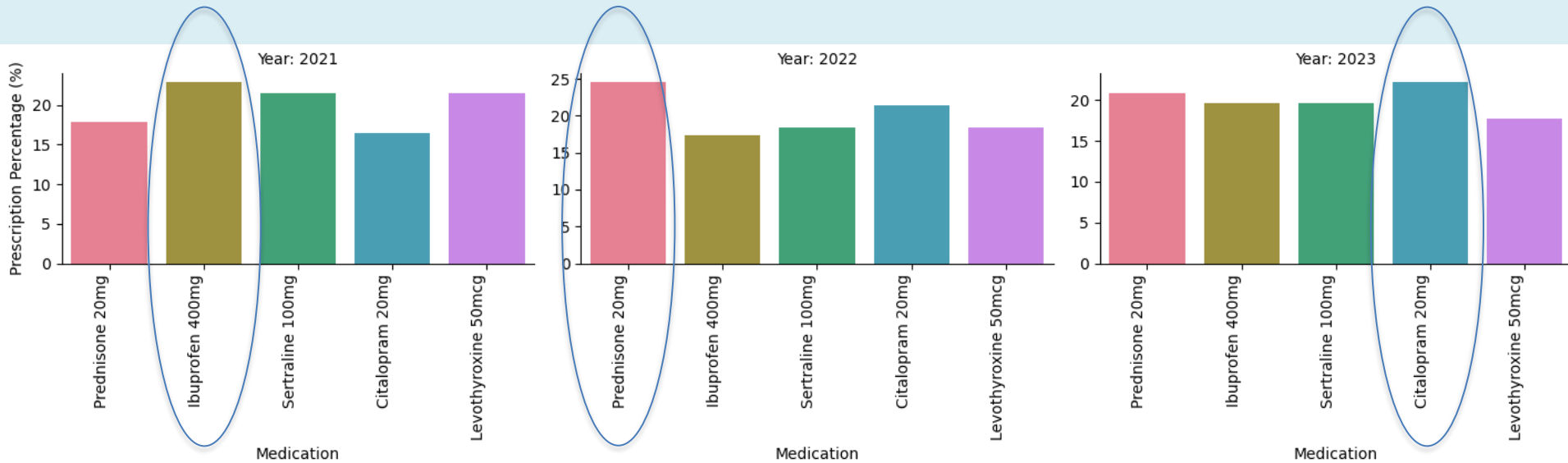


# Key Findings: Top 5 Most Frequent Medicines





# Key Findings: Top 5 Most Frequent Medecines (over years)

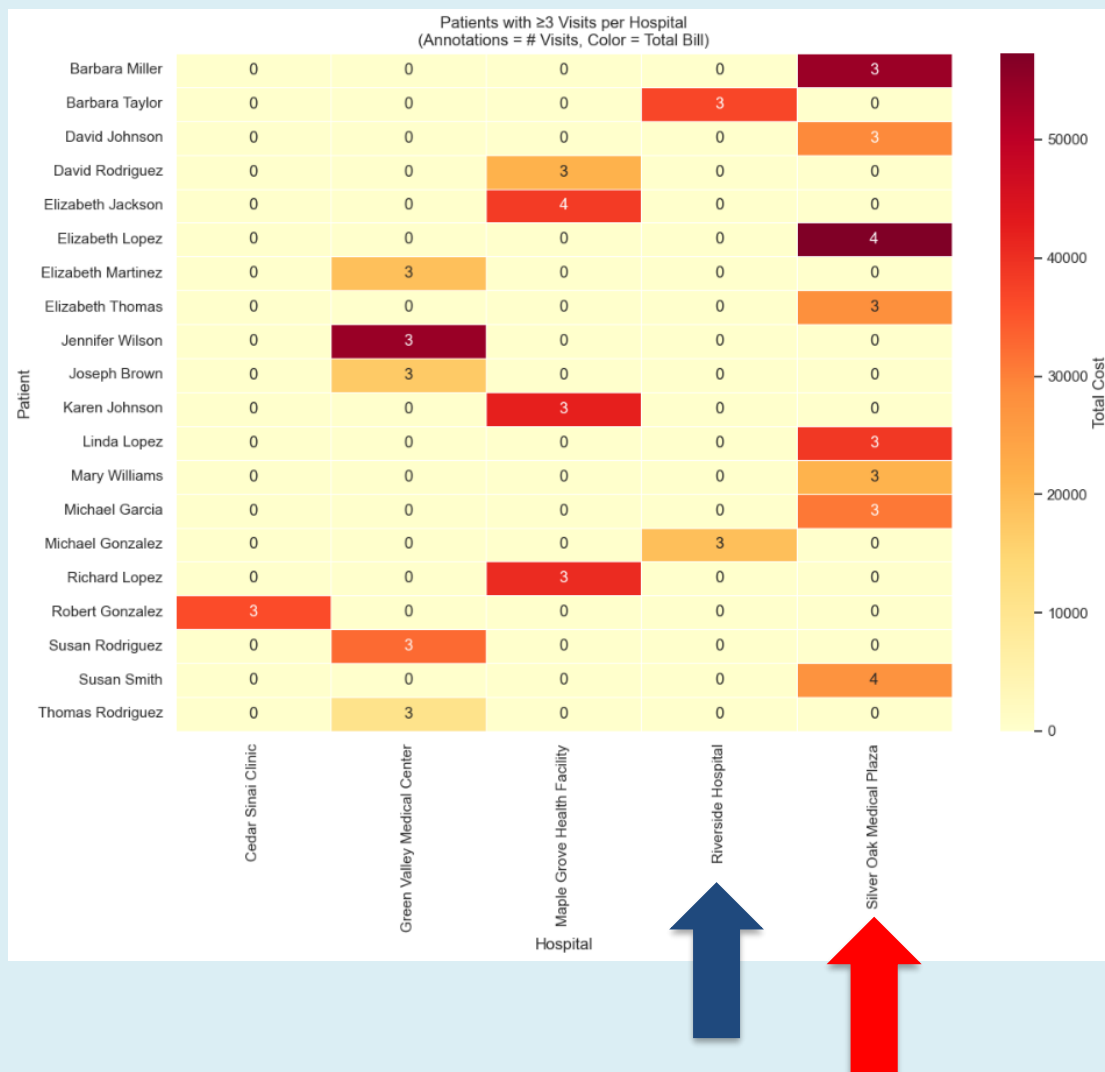


YEAR 2021 – Ibuprofen 400 mg

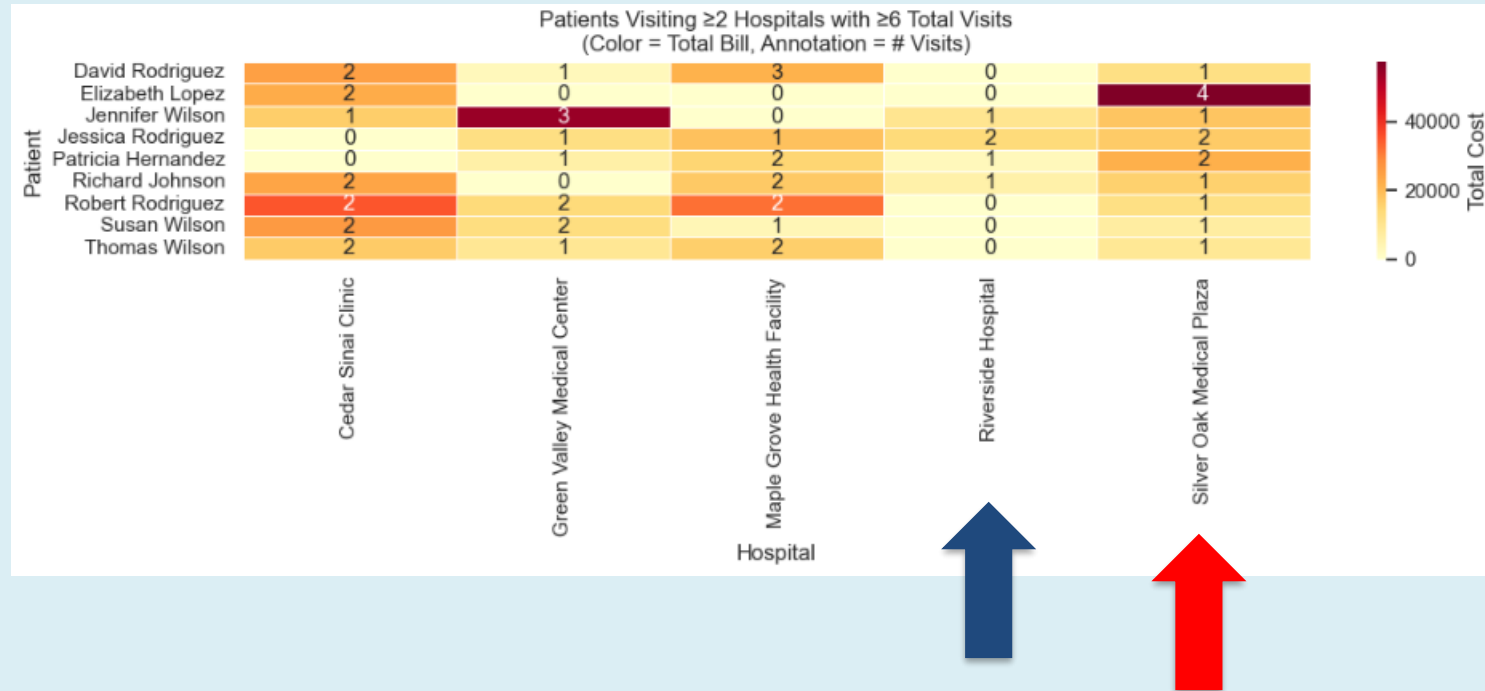
YEAR 2022 – Prednisone 20 mg

YEAR 2023 – Citalopram 20 mg

# Key Findings: Patients with recurrence (I)



# Key Findings: Patients with recurrence (II)



David Rodriguez – 7 visits in 4 hospitals

Robert Rodriguez – 7 visits in 4 hospitals

# Conclusions

- Successful Data Cleaning & Transformation
- Basic Text Information Extraction (“Full prescription Details”)
- Successful Exploratory Data Analysis
- Poor Identification of Clinical vs Economic Insights
  - Most variables do not show significant relationship with cost
  - Only Age and Diagnosis exhibit statistically relevant differences
  - Elderly patients with chronic diseases account for the majority of hospital visits
  - Preliminary analysis shows a decline in hospital visits in 2023
  - Steady growth observed in mental health medication prescriptions
  - Patient repeat visits vary by hospital
- Main limitation is the synthetic nature of dataset

# Future Work

- Apply Machine Learning Models to:
  - Predict TotalBill
  - Predict RecoveryRating
- Use more advanced NLP techniques for
  - Prescription analysis
- Include additional variables:
  - Comorbidities (ie. Diabetes, Hypertension)
  - Laboratory results (i.e glucose, creatinine...)
- Validate findings on real-world healthcare datasets

# *Thank you*

*"You lose your curiosity when you stop learning"*

*Katherine Johnson- Physicist, Space Scientist and Mathematician*